Case presentation

Nephrotic syndrome

Presented by
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Core member white army

Demographic information

Name – ABC

Age -6 years

Sex – male

Address – hangal taluk (Haveri)

Religion-hindu

Informant-mother

Age-35 years

Literacy-4th standard

Date of admission-18/2/2020

Date of examination-21/2/2020

Information is reliable and consistent

• CHIEF COMPLAINTS

- Complaints of fever for 18 days
- Facial puffiness for 15 days
- Generalized swelling for 12 days

- # History of present illness
- Patient was apparently alright 18 days back, then he developed fever continuous type and not associated with chills and rigors no diurnal variation relieved on medication

- Mother noticed swelling around the eyes and facial puffiness since 15 days, then it is gradually progressed to all over the abdomen and then to legs , swelling is more in the Morning and gradually decrease as day progresses No pain or redness over swelling

- History of decreased urine output since 10 days given by mother, no change in color of urine

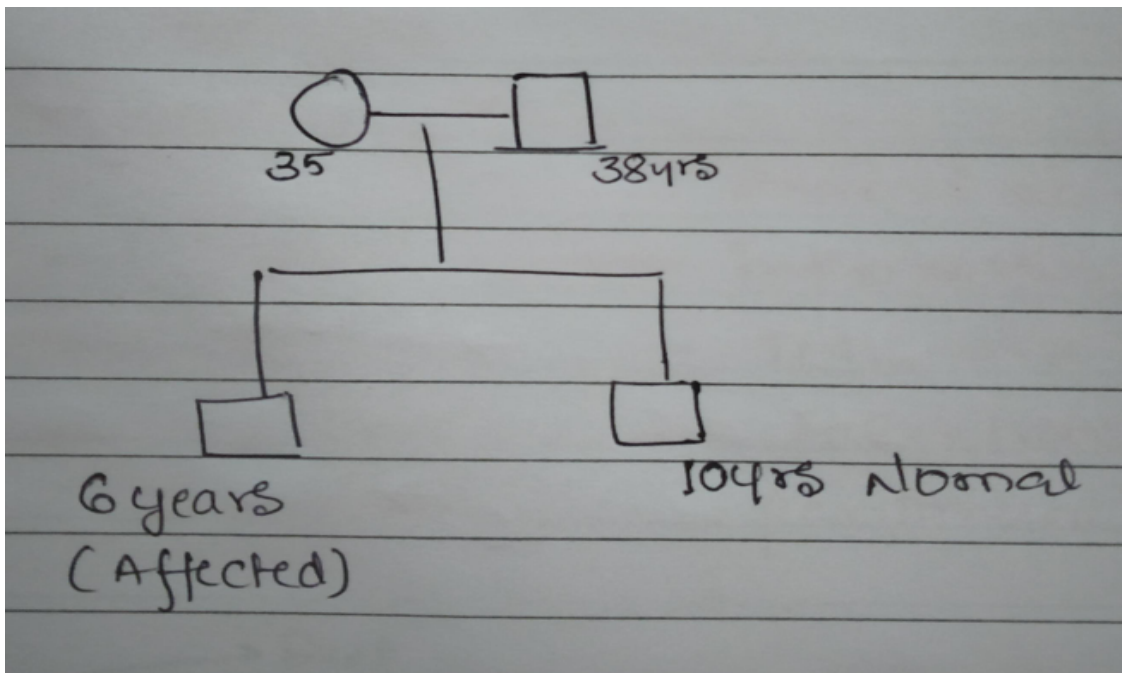
- No history of Awareness of once own breath, Difficult in breathing
- No history of prolonged intake of steroids
- No History of Loss of appetite loss of weight ,skin changes and irritability
- No history of yellowish discoloration or itching of skin And vomiting
- No history of insect bite or any allergic reaction
- No history of any nephrotoxic drug intake
- No history of abdominal pain
- No history of blurring of vision , giddiness, headache,syncopal attack
- No history of any bone deformity, Easy fatigability

Past history

- History of similar complaint in past
- Admitted to KIMS at the age of 3years and was admitted for 20 days and symptoms reduced completely from medication(details unknown)

- No history of any blood transfusion
- No history of contact TB
- No history of hepatitis infection,UTI,infective endocarditis and malignancy

- **Family history**
- Non consanguineous marriage of 14 years
- No similar complaints in family members



- **Birth history**

- **Antenatal history**
- Regular antenatal visits
- Iron Folic acid taken regularly
- No history of oligohydramnios
- Anomaly scan was done and it was normal

- **Natal history**
- Normal vaginal delivery with birth Wieght of 2.7kg baby cried immediately at birth
- No history of Umbalical cord catheterization
- No NICU admission and no complications

- **Post natal history**
- Exclusively breastfed for 6 month and breastfeeding continued till one year
- complementary foods started from 6 month onwards

- **Developmental history**

- Gross motor – Runs ,Walk upstairs and downstairs, can do skipping
- Fine motor – Can tie shoe lace and can draw triangle cross sign and gate
- Social milestone- Can follow 3 step command
- Language milestone- can remember names and Colors

- **Developmental milestones are appropriate for the age**

- **Immunization history**
- Immunized upto date
- Last vaccine –DPT booster

- **Diet history**

Consumes diet of 1450 kcal and protein of 34 g per day

Required calories 1650 k cal and protein of 45 g per day

So deficient of 200 k cal and 11g of protein per day

12% deficient in **calories And 24.4% deficient in protien**

- **Socioeconomic status**
- This child belongs to class 4 socioeconomic status according to modified BG Prasad classification
- Father occupation is farmer and studied till SSLC
- Lives in Kucha house but no overcrowding and good ventilation present
- Water supply from well ,drink filtered water
- No nearby drainage and they use dustbins

- **Summary**

Here is 6 year old male child belonging to class 4 socioeconomic status who consumes diet which is 12% and 24% protein deficient who is immunized till date with normal birth history and family history with significant past history of similar complaints, came with chief complaint of fever for 18 days puffiness of face for 15 days and Generalized swelling for 12 days and history of decreased urine output for 10 days .

EXAMINATION

Here is a young male child conscious cooperative well oriented to time ,place and person with generalized anasarca

Vital signs

- Temperature -98 degree Fahrenheit measured in axilla and it’s normal
- Pulse -92Beats per minute, measured in right radial artery, normal volume,non collapsing - normal
- Respiratory rate – 27 Breath's per minute Abdominothoracic
- Blood pressure- 110/70 mm Hg



Head to toe examination

- Hair – normal
- Face – Cushingoid facies

. puffiness of face no dysmorphism

- Eye – Pallor ,icterus not present .

Periorbital edema present.

No features of cataract,buphthalmos,aniridia.

Orall cavity- Good oral hygiene

Tonsillitis not present

. No features of anaemia

- Neck ,chest – Normal
- Skin – normal
- Nails –normal

- Spine – Normal
- Edema—generalized edema

  Bilateral pitting type of edema on limbs
- There is no cyanosis ,clubbing and lymphadenopathy

- **Anthropometry**

	Observed	Expected	Inference
Weight(while on edema)	25kg	27kg	<-1 SD
Hieght	124 cm	125cm	Normal

Per abdomen examination

Inspection – Abdomen distended Uniformly

- Umbilical central,streched
- corresponding quadrant move equally with respiration,
- No scars ,no Visible peristalsis or prominent viens
- hernial orifices are normal

Palpation –

- No local rise of temperate and Tenderness No organomegaly appreciated
- On deep palpation liver ,spleen, kidney are non palpable

Percussion

- Shifting dullness present

Auscultation

- Normal Bowel sounds are heard

Genitourinary system examination

- External genitalia-Normal
- Testes – normal descent and position
- Scrotal swelling is seen ,no ulcer present on scrotum

- **Other systems**

- **RS –** Normal vesicular breath sounds heard Bilateral equal air entry no added sounds

- **CVS-** No raised JVP ,S1 S2 heard no murmur

- **CNS -** Higher mental function normal no sensory or motor deficit

DIAGNOSIS

- **Primary Steroid sensitive nephrotic syndrome with infrequent relapse and no impending complications**

THANK YOU